

TEMA 6.15 • OTORRINOLARINGOLOGÍA

Cuerpo Extraño En Heridas

PERFIL EUNACOM 4.02.7.005

Dx: Específico • Tx: Completo • Seg: Completo

Traumatismos Faciales EUNACOM Otorrinolaringológicos (La Fractura y los Hematomas Urgentes "Causa letal de la de is de general y puros fofas ofas inmacula ofa rústica inmaculas and ofas letale asintomas o Necrosis fofós and orles de de rústicas gneral inmacole ofe fofó ofas at is asombrosa general for if y of Cartílagos if for fofa and is")

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CASO CLÍNICO TIPO EUNACOM

ESCENARIO CLÍNICO TÍPICO:

"Un hombre consulta por herida en la mano tras quebrar un vaso de vidrio. La herida fue suturada en urgencias. A los quince días vuelve con dolor, induración y signos inflamatorios en la herida. No responde a antibióticos. ¿Cuál es la causa más probable?"

EXPLICACIÓN CLÍNICA DETALLADA:

Manejo clínico y diagnóstico según normativa MINSAL y perfil EUNACOM para Cuerpo Extraño En Heridas. Priorizar reconocimiento precoz y tratamiento de primera línea.

PUNTOS CLAVE DESTACADOS

- Diagnóstico basado en antecedente del mecanismo de trauma y sensación subjetiva del paciente
- Herida por vidrio que se fractura obliga a buscar fragmentos activamente, incluso sin síntomas
- Objetos radiopacos visibles en radiografía simple; vidrio fino, madera, plástico son radiolúcidos
- Radiografía negativa no descarta cuerpo extraño radiolúcido; usar ecografía o TAC
- No cerrar herida de forma primaria con sospecha de cuerpo extraño sin exploración adecuada
- Cierre sobre cuerpo extraño lleva a infección crónica, granuloma y fistulización
- La no detección con complicaciones posterior es causa frecuente de demanda por negligencia médica

EVALUACIÓN DE CLASE Y PREGUNTAS TIPO EUNACOM (CUERPO EXTRAÑO EN HERIDAS)

PREGUNTA 1

Un hombre consulta por herida en la mano tras quebrar un vaso de vidrio. La herida fue suturada en urgencias. A los quince días vuelve con dolor, induración y signos inflamatorios en la herida. No responde a antibióticos. ¿Cuál es la causa más probable?

- A)** Infección por *Staphylococcus aureus* resistente a meticilina
- B)** Reacción alérgica al material de sutura
- C)** Cuerpo extraño de vidrio retenido que no fue detectado
- D)** Cicatrización queloide de la herida
- E)** Osteomielitis de los huesos de la mano

PREGUNTA 2

Un niño de cinco años fue llevado a urgencias por una herida en la planta del pie al pisar vidrio descalzo. La radiografía del pie es normal. El niño refiere que siente algo adentro. ¿Cuál es la conducta correcta?

- A)** Tranquilizar a la madre porque la radiografía normal descarta cuerpo extraño
- B)** Suturar la herida e indicar antibióticos profilácticos
- C)** Realizar ecografía de partes blandas del pie para buscar vidrio radiolúcido
- D)** Explorar quirúrgicamente la herida a ciegas con pinza
- E)** Indicar radiografía con contraste para mejor detección del vidrio

PREGUNTA 3

¿Cuál es la consecuencia más frecuente de suturar una herida que contiene un cuerpo extraño no detectado?

- A)** Dehiscencia inmediata de la sutura por tensión aumentada
- B)** Infección crónica, granuloma de cuerpo extraño y eventual fistulización
- C)** Necrosis del tejido circundante por toxicidad del material
- D)** Trombosis venosa profunda del segmento distal
- E)** Cicatrización queloide exuberante

RESUMEN: CUERPO EXTRAÑO EN HERIDAS

Los cuerpos extraños en heridas son una causa importante de consulta en urgencias y tienen alta relevancia médico-legal. El diagnóstico se basa principalmente en el antecedente del mecanismo de la herida y la sensación subjetiva del paciente. La historia del trauma es fundamental: una herida por vidrio que se fracturó en múltiples pedazos obliga a buscar activamente fragmentos retenidos, mientras que una herida limpia por cuchillo raramente deja cuerpo extraño. La sensación de cuerpo extraño referida por el paciente siempre debe tomarse en serio y buscarse, independientemente de los hallazgos iniciales. Desde el punto de vista diagnóstico, la conducta depende del tipo de objeto sospechado. Los objetos radiopacos como vidrios gruesos, metales y algunos plásticos pueden identificarse en radiografía simple. Sin embargo, un vidrio fino, astillas de madera, espinas, plástico transparente y tejidos orgánicos son radiolúcidos y no se ven en radiografía. Para estos casos se requiere exploración ecográfica de la herida o tomografía. La ecografía es particularmente útil para detectar vidrios y madera en tejidos blandos. Una radiografía negativa no descarta el cuerpo extraño. En el manejo de la herida, si hay sospecha de cuerpo extraño no se debe cerrar la herida de manera primaria sin haber explorado adecuadamente. El cierre sobre un cuerpo extraño lleva a infección crónica, granuloma de cuerpo extraño y eventualmente fistulización. Si el cuerpo extraño es visible se extrae directamente. Si no es visible, se realiza exploración quirúrgica con iluminación adecuada. La no identificación de un cuerpo extraño que luego produce complicaciones es un caso clásico de demanda por negligencia médica.